When I took over Eastcom, I was aware that my schedule would take me from Kolkata at sea level to these HAA and super HAA areas. With my medical history, I felt it would be prudent to undergo a full medical, especially for my abnormal ECG. At the Command Hospital (CH), I was again put through a battery of tests and this time, a name was given to my condition: Apical Hypertrophic Cardio Myopathy (AHCM), a rare medical condition, first discovered in Japan in 1976.[5] The cardiologist at the CH was extremely concerned and suggested various lines of treatment, including installing a pacemaker. The Commandant, Maj Gen Sunil Kant, suggested we take a second opinion from a renowned cardiologist in Kolkata, Dr Rabin Chakravarty of the Medica Super Speciality Clinic, who specializes in heart rhythm disorders. We went to him and presented all the facts and the recommended line of treatment. Dr Rabin made a very telling remark, that in the Army, doctors treat a patient looking at his rank rather than the symptoms. It was a remark that was to stick with me. He clarified that he had seen many such cases in people from all walks of life, and that it is not associated with sudden cardiac death and has a benign prognosis. Since this was a totally different prognosis, we decided to take yet another opinion, this time from AIIMS, Delhi. Here too, Dr Bisoi and Dr Sanjeev, after reviewing all the reports, said that there was nothing to worry about, but that I should avoid any sudden or extreme physical exertion. Basically, the same thing that the cardiologist at MH, Meerut, had told me in 1995, two decades earlier. It has always been my policy to make health a priority and to take medical treatment as appropriate. I used to make this point in my address to the officers not to hide any underlying medical condition and that it was better to be 'a living Major rather than a deceased Major General'; not to conceal medical problems for the sake of promotions.

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